A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

13 May 2026

Dr Catherine Falconer
Eastbrooke Family Clinic Lincolnville

Dear Dr Falconer,

Re: Ms Irene Crawford
DOB: 12/06/1960 Phone: 0417 574 193
Address: 42 Victory Street, KEILOR PARK, VIC, 3042
Email: irenecrawford2014@hotmail.com

Summary

- **Sessile serrated adenoma completely excised.**
- **Mild symptoms of eosinophilic oesophagitis.**
- **For combined surveillance gastroscopy and colonoscopy in three years.**
- **No action required by you.**

I had the pleasure of conducting a telehealth review with Irene following her recent colonoscopy.

Irene's colonoscopy revealed a sessile serrated adenoma. This polyp was completely removed, and I have reassured her that there were no features of malignancy. This finding is similar to her previous surveillance history where she had a different type of pre-cancerous polyp with similar growth potential. Although international guidelines vary, under current Australian clinical guidelines, this finding necessitates a surveillance colonoscopy in three years' time.

Clinically, Irene describes a sensation of "heaviness" after eating, but notably, she does not suffer from regular indigestion or food bolus obstruction. Given that her symptoms are currently quite mild from an eosinophilic oesophagitis (EoE) perspective, I do not believe there is significant value in commencing a highly restrictive elimination diet at this stage.

Impression and Plan I have opted to defer any specific dietary or pharmacological intervention for her EoE until such time as her symptoms become more clinically significant. We discussed the role of dairy as a common trigger for this condition, and she is supported by a dietitian in this regard. The plan is for Irene to undergo a combined surveillance gastroscopy and colonoscopy in three years' time. We will send her a reminder closer to the date. I have advised her to contact me earlier for a review should she develop any red-flag symptoms, such as haematochezia or an escalation in her dysphagia.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
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Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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CC: Dr Leah Curtis

Investigation Results:

Crawford, Irene : HBEC 326133, HHS 164992

DOB: 12/06/1960 Age: 65 years

42 Victory St, Keilor Park 3042

Results from 23/04/2026 ..... Episode# 26P300097 Test Hist (Histology - Routine)

FINAL ANATOMICAL PATHOLOGY AAnat

Lab #: 2026B010574 **CLINICAL NOTES** TV polyp.? SSA. **MACROSCOPIC DESCRIPTION** Specimen labelled "Transverse Colon Polyp". Three tan polypoid fragments measuring 2 x 1 x 1 mm to 4 x 3 x 1 mm. Processed whole. A1. **MICROSCOPIC DESCRIPTION** The sections of colonic mucosa show areas of serrated luminal architecture with dilated glands extending to crypt bases. There is no dysplasia or malignancy. **PATHOLOGIST'S OPINION** **Transverse colon polyp: Sessile serrated lesion/polyp, without dysplasia.** Pathologist: Dr Rohini Joshi

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